

Artwork Document Information		Artwork Type [Please Tick /]		Sign / Stamp	
Product : T - UPHADYL PAED EXPECTORANT SAP No. : Date : 19 May 2025		☐ Unit Box ☐ Outer Box ☐ Label ☒ Leaflet ☐ Fix-A-Form ☐ Shipper Carton ☐ Aluminium Foil ☐ Sachet ☐ Others : _____			
Document Category [Please Tick /]			Pharmacode / QR Code / Others Info		
☒ RA/NPRA Artwork ☐ Internal Artwork Circulation ☐ Others : _____ ☐ Recirculation Artwork ☐ Previous Approved Artwork ☐ Approval Copy Artwork ☐ Draft Artwork ☐ Clean Copy					
Remarks		Colour			
1. Font : Helvetica Regular (Spacing - 8.5pt / Size - 7pt) 2. Dimension : 148.5mm(w) x 210mm(h)		Black			
Colours shown on this proof are to indicate correct colour separations.					

210 mm

UPHADYL paed EXPECTORANT

RELIEVES CHESTY COUGH

DESCRIPTION

Brown, flavoured and homogenous syrup.

COMPOSITION

Each 5ml contains:

Ammonium Chloride 67.5mg

Diphenhydramine Hydrochloride 7.0 mg

PHARMACODYNAMICS

The combination of Diphenhydramine Hydrochloride, an antihistamine and Ammonium Chloride, an expectorant, will provide rapid relief for cough which is associated with common cold influenza or allergic conditions. The menthol will give a soothing effect.

PHARMACOKINETICS

DIPHENHYDRAMINE HYDROCHLORIDE

Diphenhydramine hydrochloride is well absorbed from the gastro-intestinal tract, though high first-pass metabolism appears to affect systematic availability. Peak plasma concentrations are achieved about 1 to 4 hours after administration by mouth. A mean peak plasma-diphenhydramine concentration of 110 ng per mL has been reported after administration of 50 mg four times daily by mouth. Diphenhydramine is widely distributed throughout the body including the CNS. It crosses the placenta and has been detected in breast milk. Diphenhydramine is highly bound to plasma proteins. Metabolism is extensive. Diphenhydramine is excreted mainly in the urine as metabolites; little is excreted as unchanged drug. Excretion is almost complete within 24 hours of administration.

AMMONIUM CHLORIDE

Ammonium chloride is absorbed from the gastro-intestinal tract. The ammonium ion is converted into urea in the liver; the anion thus liberated into the blood stream and extracellular fluid causes a metabolic acidosis and decreases the pH of the urine; this is followed transient diuresis.

INDICATIONS

For cough and compilation associated with allergic disorders and allergic manifestation of respiratory illness e.g. common cold, Flu, and Bronchitis.

CONTRAINDICATIONS

Hypersensitivity to any of the ingredients. Patients receiving MAOIs.

SIDE EFFECTS/ADVERSE REACTIONS

The most common side effect is sedation.

Other side effects include gastric intestinal disturbances such as nausea, vomiting, diarrhea or constipation and epigastric pain. It may produce headache, blurred vision, tinnitus, elation or depression, irritability, nightmare, anorexia, difficulty in micturition, dryness of the mouth, tightness of chest and tingling, heaviness and weakness of the hands.

PRECAUTIONS/WARNINGS

Uphadyl paed may enhance the sedative effect of central depressant including alcohol, barbiturates, hypnotics, sedative and tranquilisers.

It may cause drowsiness and dulling of mental alertness. Patients undergoing treatment should not take charge of vehicles, other means of transport and machinery where loss of attention may lead to accidents.

When used for treatment of cough and cold:

(a) Not to be used in children less than 2 years of age

(b) To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age

INTERACTIONS WITH OTHER MEDICAMENTS

There has been a report suggesting that a reduction in temazepam, metabolism caused by diphenhydramine may have contributed to perinatal death after ingestion of these drug by the mother.

PREGNANCY AND LACTATION

A pregnant woman who has receiving diphenhydramine hydrochloride 150mg daily for pruritic rash gave birth to an infant who developed diarrhoea and generalised tremulousness 5 day later. The delay in appearance of withdrawal symptoms was considered to be due to absence of full activity of glucuronyl conjugating enzymes in the first few days of life.

ROUTE OF ADMINISTRATION: Oral

DOSAGE

Children above 2 years: 5 to 10 ml 3 or 4 times daily.

OVERDOSAGE

In case of accidental overdosage, professional assistance must be considered or poison control centre should be contacted immediately.

PRESENTATION

60ml. Not all pack sizes are available locally

STORAGE CONDITIONS AND USER INSTRUCTIONS

Store in a dry place below 30°C. Protect from light.

Keep out of reach of children.

Jauhi daripada kanak-kanak.

Shake well before each dose. Keep container tightly closed

Shelf life : Please refer to outer package.

Product Registration Holder:

Duopharma Manufacturing (Bangi) Sdn. Bhd.
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Bangi Industrial Estate, 43650 Bandar Baru Bangi,
Selangor, Malaysia.

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Revision Date: May 2025



148.5 mm